



Community Mental Health Service (CMHS) Provision: Responding to Respiratory Virus Illness SOP

Scope

Site	Department, division, or operational area	Applicable to
Royal Perth Bentley Group (RPBG)	Mental Health (MH) Division, Community Mental Health Services (CMHS)	All staff

Purpose

This standard operational procedure (SOP) aims to guide clinical practice for management of consumers of Bentley, City East, Wungen Kartup and Midland CMHS with suspected or confirmed acute respiratory virus infection (ARI).

Links within document

- [Appendix I: Response to crisis/unplanned clinic presentation of known/unknown consumer](#)
- [Appendix II: Out-of-hours Bentley Health Service \(BHS\)](#)
- [Appendix III: Responding to RV Illness: CMHS vehicle transport guidelines](#)

Key points

- Respiratory viruses are those that can affect the breathing passages causing respiratory illnesses ranging from the 'common' cold to influenza and influenza like illness and COVID-19
- **All CMHS consumers are to be screened upon presentation** to assess whether they have ARI. Additionally, screening as warranted by onset of symptoms whilst attending CMHS
- Consumers presenting with symptoms of ARI should be managed using respiratory virus precautions and encouraged to wear a surgical mask unless clinically inappropriate. Respiratory virus testing (rapid antigen testing [RAT] or polymerase chain reaction [PCR]) is not necessary to confirm infection. A negative test does not remove requirement for respiratory virus
- CMHS have different clinic layouts, and it is important that staff access individual site plans for walk through processes to facilitate the management of consumers with suspected or confirmed ARI. For site plans, refer to local emergency procedures
- The guiding principles for service provision to consumers will be governed by policies, guidelines and other documents as outlined in the below links. All consumers will be provided mental health (MH) care as required, regardless of vaccination status. Refer to **Respiratory Virus Illness Prevention and Management Clinical Guideline** and **Community Mental Health Services Triage to Discharge SOP**.

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Version: 3.0
Endorsed by: Mental Health Division Co-Director 03/11/2025
Authorised by: Clinical Practice Committee 04/12/2025

Date Compiled: 03/2022
Executive Sponsor: Mental Health Co-Director
Revision Due: 12/2029

Procedure details

Screening

- Upon entry to the CMHS, reception staff are to:
 - Inform all consumers that ARI screening is required
 - Ask consumers the following screening questions:
 - Do you have any cold or flu symptoms? (see below symptoms)
 - a cough
 - breathing difficulty
 - sore throat
 - runny nose (rhinorrhoea) or nasal congestion
 - which may or may not be accompanied by a headache or fever (greater than or equal to 37.5°C)
 - Encourage consumers with ARI symptoms to wear a surgical mask and perform hand hygiene when necessary; consider some consumers may have an exemption regarding mask wearing or may experience increased psychological distress as a result
 - Staff who cannot maintain minimal 2 metres, distance must don respiratory precautions PPE
 - Document as per local processes, i.e. a log or calendar spreadsheet of consumers who present to CMHS, with a yes or no tick box: indicate screened
- The Treating Clinician should be informed of consumers who have been identified with symptoms of RV or COVID-19 household contacts and those who are unable or unwilling to wear a mask so that respiratory virus precautions can be implemented
- Refer to the **Respiratory Virus Illness Prevention and Management Clinical Guideline** for most current definitions
- For additional guidance for managing unknown consumers or unplanned presentations refer to Table 1 in [Appendix I: Crisis/unplanned clinic presentation of known/unknown consumer](#).

Screening out of hours

- For Midland and City East CMHS, the Assessment Treatment Team (ATT) is to complete respiratory virus screening prior to any face-to-face contact

Respiratory virus precautions

Where indicated, respiratory virus RV precautions should be used in addition to standard precautions and source control for any **consumer who has ARI symptoms** including the following:

- Healthcare workers approaching within 2 metres of the consumer should wear fit tested particulate filter respirators (PFR) and eye protection
- Disposable apron or gown and gloves if required for standard precautions, including risk of exposure to blood or bodily fluids, or risk of uniform being contaminated by significant physical contact with a consumer
- Consumer to be encouraged to wear a surgical mask, practice respiratory etiquette, and perform hand hygiene after handling their mask or touching face or after respirator etiquette
- Consumer should be encouraged to leave the CMHS as soon as possible after the consultation
- Equipment or furniture used by the consumer should be cleaned and disinfected prior to use by another consumer (use combined detergent disinfectant wipes).

Testing

- RAT or PCR is not necessary to confirm infection for CMHS consumers presenting with ARI symptoms A negative test does not remove the requirement for respiratory virus precautions.

Consumer care provision: face-to-face appointments

Clinicians are to undertake the following:

- Determine care requirements of consumers
 - Review all active consumers and identify the most suitable service provision modality and document the outcome in the consumer's *Treatment Support Discharge Plan (TSDP)* or Stay Strong Plan (Wungen Kartup only)
- Consider telehealth where clinically indicated
 - Complete RV-related risk assessment including ask the consumer's vaccination status and document in the consumer's notes
 - To arrange telehealth appointments, refer to Health Direct and the Team leader or the nominated telehealth champion at each site for any technical support or equipment support requirements. Refer to [EMHS Virtual Care \(Telehealth\) Toolkit on HealthPoint](#).
- Routinely encourage or support those consumers who have not yet been vaccinated to access vaccination

Consumer care provision: face-to-face appointments cont'd

- In assessments of consumers with ARI the following should be provided:
 - PPE station with PRF respirators and eye protection. PPE to be donned prior to consumer entering the interview room, or when at least 2 metres from the consumer
 - Surgical mask to be offered consumer
 - Combined detergent-disinfectant wipes
 - Alcohol-based hand rub
 - Furniture that can be cleaned and disinfected (non-cloth material) using combined detergent – disinfectant wipes
 - General waste bin without a lid and lined with black plastic bag for disposal of PFR and other PPE if required. Dispose of rubbish bags in general waste. A clinical waste bin is not required
- **Only if ARI is indicated**, the following practices should be adhered to within the interview room
 - Staff to perform hand hygiene according to the 5 Moments for Hand Hygiene, and additionally as required to prevent contamination of items that cannot be easily cleaned e.g. healthcare record
 - Encourage the consumer to wear a surgical mask, and discourage activities that require mask removal e.g., consumption of food or drink while in clinic
 - Staff who cannot maintain minimal 2 metres, distance must don respiratory precautions PPE
 - Where possible, encourage consumer to perform hand hygiene when entering and exiting the room
 - When the consumer leaves the room, use detergent-disinfectant wipes to clean and disinfect items handled by the consumer, including pens, telephone, blood pressure machine, and their chair. Further cleaning of the room is not required
 - Staff to use detergent-disinfectant wipes to clean and disinfect eye protection before returning to PPE
 - If gross soiling occurs, call Patient Support Services (Bentley only) for cleaning room, alternatively it may be done by designated staff members

Compliance monitoring

Team leader or Program Manager to monitor:

- Datix Clinical Incident Management (CIM) system process and/or Combined Hazard or Incident Report (CHoIR) form
- Compliments/Complaints Management System.

Facilitator

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Related policies, practice standards, clinical guidelines

Policies and procedures SharePoint

- [Respiratory Virus Illness Prevention and Management Clinical Guideline](#)
- [Community Mental Health Services Triage to Discharge SOP](#)
- [Community Mental Health Home Visiting SOP](#)
- [Transmission-Based Precautions: Use of Personal Protective Equipment \(PPE\) SOP](#)
- [Visitor Management Policy](#)
- [Recognising and Responding to Acute Deterioration in Mental State, Cognition and Behaviour SOP](#)
- [Hand Hygiene Policy](#)

EMHS Policies

- [Prevention and Management of Workplace Aggression and Violence Policy](#)
- [Community/Home Visiting Policy](#)
- [Transportation of Mental Health Consumers Policy](#)

WA Department of Health Policy Hub

- [Code of Practice for Clinical and Related Waste Management](#)
- [State-wide Standardised Clinical Documentation for Mental Health Services](#)
- Motor Vehicle Fleet Policy

Other

- [Mental Health Act 2014](#)
- [Charter of Mental Health Care Principles \(MHA 2014\)](#)
- [RPBG Infection Prevention and Management on HealthPoint](#)
- [EMHS Virtual Health \(Telehealth\) Toolkit on HealthPoint](#)
- [Healthy WA COVID-19 information](#)
- [Work, Health and Safety – Chemical Management on HealthPoint](#)
- [Coronavirus \(COVID-19\) environmental cleaning and disinfection principles for health and residential care facilities – Department of Health.](#)

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Related national standards

ACSQHC NSQHS Standards 2nd Edition (2021):

Standard 1: Clinical Governance.

Standard 3: Preventing and Controlling Infection

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Appendix I: Crisis/unplanned clinic presentation of known/unknown consumer

If a consumer presents to a CMHS unexpectedly or in crisis and is either known or unknown to staff, Reception staff are to:

- Conduct the current ARI screen, where possible. Refer to the [Screening](#) (above)
- Contact the Duty Clinician when experiencing difficulties with consumers or visitors relating to ARI screening
- Refer to Table 1 for further guidance

Table 1: Response to crisis or unplanned clinic presentation of known/unknown consumer

Consumer	Staff Responsible	Actions to be taken
If consumer is unknown and screens ARI negative	Reception staff	• Alert the Duty Clinician of the outcome
	Duty Clinician	• Escort the consumer to an interview room
If the consumer is known and screens ARI negative	Reception staff	• Alert the Duty Clinician of the outcome
	Duty Clinician	• Determine if the consumer can be redirected home and a full assessment done via telehealth or home visit (H/V) or depending on urgency, continue the assessment in the clinic with an ATT Clinician • Assess if the Care Coordinator can make contact via telehealth or H/V, if needing urgent review or assistance • Conduct assessment with Care Coordinator or Treating team member
If the consumer is known or unknown and has symptoms of ARI	Reception staff	• Alert the Duty Clinician of the outcome
	Duty Clinician	• Don or wear PPE as directed by Respiratory Virus Illness Prevention and Management Clinical Guideline • Direct consumer to the designated clinical/ interview room ○ If a consumer is unwell and requests a RAT, provide a RAT • Depending on the consumers mental state, determine if an urgent assessment needs to be undertaken and proceed to complete full assessment with assistance from ATT clinician or consumer's Care Coordinator or Treating Team member, wearing PPE as directed by Respiratory Virus Illness Prevention and Management Clinical Guideline

Appendix II: Out-of-hours Bentley Health Service (BHS)

BHS consumer self-presentation pathways for assessment

During business hours 08:30 – 16:30, Reception staff or clinicians are to:

- Ask the ARI inclusion criteria screening questions for consumers prior to entry to clinical area. Refer to **Respiratory Virus Illness Prevention and Management Clinical Guideline**
- Clinicians are to review the consumer's current ARI RV infection status against inclusion criteria screening questions and presenting mental health issues
- Determine the appropriate location of assessment and wear PPE based on consumer ARI issues present following screening questions (clinicians only)
- consider use of the Assessment Treatment Team (ATT) high security interview room, outdoor, garden waiting area if a consumer refuses to wear a mask

Out of business hours 16:30 – 22:00, ATT clinicians are to:

- Prioritise the ATT clinical demands across both self-presentations and urgent, community outreach assessments based on presenting clinical issues for the consumer
- If no ATT staff onsite at the time, the Duty Clinical Nurse Specialist (CNS) in mental health inpatient services together with the Duty Medical Officer (DMO) are to undertake a review
- Ensure that all self-presentations are to occur at the F Block front entrance
- Based on the presenting mental health issues and associated risks following a dynamic risk assessment to inform primarily staff safety, determine the need for Security to be present prior to entry into the F Block Triage area
- Enter F Block area via back pathway to minimise direct face to face contact with the consumer before screening occurs depending on the level of acuity
- Conduct initial ARI screening at the afterhours triage window which may influence the risk assessment or management plan
- If known or ARI issues present following screening, encourage, or check surgical face mask is provided
- Determine the location to complete a comprehensive consumer assessment based on multiple factors to decide the safest option for staff or others
- Escalate to the DMO or On Call Consultant for clinical concerns or the management of consumer care when there are issues that cannot be resolved
- Call the BeSAFE Team (BHS Safety For Everyone Team) if there are consumer physical healthcare concerns (Bentley CMHS only).

Appendix III: Responding to RV illness: CMHS vehicle transport guidelines

- Staff are to access leased or fleet vehicles suitable for a maximum of 3-person occupancy, to support distancing where recommended, when transporting mental health consumers if required (refer to the **WA Department of Health Motor Vehicle Fleet Policy**)
- Other vehicles should only be considered for 2-person occupancy where distancing is recommended

Hospital fleet vehicles are not to be used for transportation of consumers who have confirmed ARI

- Ensure spatial seating separation at a minimum distance of 1.5m from the driver while in the vehicle. It is recommended that vehicle seats of vinyl fabric or leather are used to ensure effective cleaning and if unavailable, use a waterproof cover on fabric seats
- All staff are required to complete the vehicle use log and maintain general cleanliness of the vehicle.

Vehicle transport process

Apply existing safety precautions and systems to support safety and transport requirements as per:

- **EMHS Community/Home Visiting Policy** and **EMHS Transportation of Mental Health Consumers Policy**
- **Community Mental Health Home Visiting SOP**

Prior to Consumer transport

- Ensure vehicle is equipped with the following items
 - PPE
 - Interior cleaning products or wipes
 - Alcohol-based hand rub
- Care Coordinator or Treating team are to review the consumer's presenting mental health issues and complete a risk assessment to determine the consumer's indication and suitability for transport (i.e. can be safely transported)
- Screen consumer against inclusion criteria for ARI as per this SOP and **Respiratory Virus Illness Prevention and Management Clinical Guideline** and manage PPE accordingly
- If ARI indicated, consideration should be given to postponing transport, other transport modality or alternative methods of assessment e.g., telephone or telepsychiatry, based on the consumer's preference and/or current needs or risks and document the outcome in the consumer's *Treatment Support Discharge Plan* or Stay Strong Plan (Wungen Kartup only)
- Determine the vehicle occupancy requirements, as appropriate
- Become familiar with the adjustable seating arrangements and the child proof lock system on the vehicle

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Should the consumer need to bring personal belongings, allow a small amount of luggage in an appropriate place within the vehicle

- Carry operational, fully charged mobile phone, and comply with all other clinic transport off site requirements (i.e., notification of use, details of consumers, duress pendant acquisition, etc. Refer to the **WA Department of Health Motor Vehicle Fleet Policy** and **RFBG Community Home Visiting SOP**
- If consumer meets any of the inclusion criteria for ARI and travel is required in a hospital or service vehicle (e.g., court attendance), ensure the vehicle is cleaned and decontaminated, using a two-step clean. Staff should don appropriate PPE when cleaning the vehicle.

On entry to vehicle

The driver is to:

- Wear a mask to open the rear access door of the vehicle **only if the consumer is ARI suspect or confirmed**
- Ensure the vehicle air conditioning or air setting is set to fresh air and consider driving with the windows down
- Ask the consumer to:
 - Use alcohol-based hand rub before entering vehicle
 - Remain 1.5m away from any seated staff member



For other requirements and hand hygiene measures for consumers transport vehicles refer to the [Department of Health Coronavirus \(COVID-19\) environmental cleaning and disinfection principles for health and residential care facilities](#)

During transport

- The driver must not wear gloves due to vehicle control concerns
- Maintain spatial seating separation (of 1.5m)
- Request the consumer to wear a mask and staff comply with PPE as per **Respiratory Virus Illness Prevention and Management Clinical Guideline**

Management of aggression or violence during transportation when consumer is ARI suspect.

- For de-escalation or other interventions, refer to EMHS Community/Home Visiting Policy and RFBG Community Mental Health Home Visiting SOP
- Where staff need to vacate the vehicle, the keys are to remain in the possession of the driver and the vehicle must be parked in a safe area and made safe and inoperable and call security (if an on-site incident occurs) and/or police/other emergency services for assistance, as appropriate
- If ARI suspect, maintain wearing PPE, if in use at the time
- If possible, the consumer should remain in the vehicle until police or ambulance or security attend, as appropriate
- Maintain situational awareness for safety of all parties and the general public

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Appendix III: Responding to RV illness: CMHS vehicle transport guidelines cont'd

Post transportation

- Use alcohol-based hand rub on exit of the vehicle
- The driver is to don a pair of gloves and, using combined detergent-disinfectant wipes, ensure the vehicle is cleaned and disinfected after each use, including:
 - All high touch points e.g., steering wheel, radio, dash, shift stick, indicators, door handles, seat belt clip and window wiper controls etc.
 - Wipe clean vehicle seats that are vinyl fabric or leather and for fabric seats, remove waterproof cover and wipe clean
 - Dispose responsibly of all rubbish in plastic rubbish bags in vehicles where available.